

Medications

omeprazole Oral

Medical History

Gastroesophageal reflux disease

Musculoskeletal History

Cervical spondylosis

Musculoskeletal Family History

None

Musculoskeletal Surgery

None

Surgical History

None

Social History

Smoking status - Former smoker

Chief Complaints:

1. Neck pain radiating into the right arm, 4 weeks.

HPI: This is a 45 year old female who:

Patient reports four weeks of neck pain radiating into the right arm to the thumb and index finger, associated with intermittent numbness and tingling. Symptoms began without clear trauma, possibly after a long drive. Pain worsens with neck extension and looking over the right shoulder, improves with the arm overhead. Denies weakness, gait disturbance, or bowel or bladder changes.

Vitals:

Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI BSA
08/01/25	R. Diaz, MA	130/82	74	16	98	98.4	65.0 in	150.2 lbs	25.0 1.76

Exam:

Comprehensive, Upper Extremity Neurovascular

Appearance: well nourished

Orientation: Alert and oriented to person, place, time.

Mood: mood and affect well-adjusted, pleasant and cooperative,

Right UE Pulses: normal radial pulse and good capillary refill

Left UE Pulses: normal radial pulse and good capillary refill

RUE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

LUE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

Cervical Spine

Cervical Spine Active ROM:

Flexion: 50 degrees.

Extension: 40 degrees.

Right Rotation: 55 degrees.

Left Rotation: 80 degrees.

Cervical Spine Passive ROM:

Flexion: 50 degrees.

Extension: 60 degrees.

Right Rotation: 80 degrees.

Left Rotation: 80 degrees.

Skin:

Cervical Spine: skin intact, no rashes or lesions.

Inspection:

Cervical Spine: paraspinal tenderness, no midline step-off

Strength:

Cervical Spine Flexion: Strength: 5/5, normal muscle tone.

Cervical Spine Extension: Strength: 5/5, normal muscle tone.

Stability:

Cervical Spine: Stable

Special:

Cervical Spine: Spurling: positive on the right

Tests

MRI Interpretation Cervical Spine

Diagnosis: Neck Pain, Right - M50.122

MRI Data:

Date: 08/01/2025

MRI of the cervical spine without contrast. Right paracentral disc protrusion at C5-C6 causing moderate right neural foraminal narrowing and contact with the exiting right C6 nerve root. No spinal cord signal abnormality. Impression: Right C5-C6 disc protrusion with right C6 nerve root impingement, correlating with clinical radiculopathy.

Impression/Plan:

1. Neck Pain, Right

Cervical disc disorder with radiculopathy, mid-cervical region (M50.122)

Associated diagnosis: Cervical radiculopathy (M54.12)

Plan: Prescription.

cyclobenzaprine 10 mg tablet PO

Sig: Take 1 po qhs x 2 weeks

Quantity: 14 Tablet Refills: 0

Plan: Prescription.

prednisone 10 mg tablet PO

Sig: Take 4 tabs day 1, taper by 1 tab daily

Quantity: 21 Tablet Refills: 0

Plan: Physical Therapy.

Follow up in 3 weeks

Note:

Findings consistent with right C6 cervical radiculopathy. Ordered a cervical MRI without contrast given the neurologic findings. Prescribed a short oral prednisone taper and cyclobenzaprine for muscle spasm while awaiting imaging. Discussed activity modification and cervical traction.

Staff:

Yusuf Halloran, DO (Primary Provider) (Bill Under)

Electronically Signed By: Yusuf Halloran, DO, 08/01/2025 01:00 PM CDT